

Pharmaniaga Diphenoxylate A Tablet

DESCRIPTION

White, round, biconvex tablet with Pharmaniaga icon on one side and scoreline on the other.

COMPOSITION

Each tablet contains Diphenoxylate hydrochloride 2.5 mg and Atropine sulphate 25 mcg.

ACTIONS

Diphenoxylate HCl is derivative of pethidine, but has no analgesic activity. It reduces intestinal motility and is used in the symptomatic treatment of acute and chronic diarrhoea.

When given by mouth atropine sulphate reduces smooth-muscle tone and diminishes gastric and intestinal motility.

Preparations of diphenoxylate usually contain subclinical amounts of atropine sulphate in an attempt to prevent abuse by deliberate overdosage.

Diphenoxylate is well absorbed from the gastrointestinal tract and extensively metabolised in the liver to diphenoxylate acid (difenoxin) and hydroxydiphenoxylate acid. It is excreted mainly as metabolites in the urine and bile; it may also be excreted in breast milk. Absorption may be delayed in the presence of atropine sulphate.

Atropine is readily absorbed from the gastrointestinal tract; it is also absorbed from mucous membranes, the eye and to some extent through intact skin.

Atropine disappears rapidly from the blood and is distributed throughout the body. It is excreted in the urine, partly as a metabolite. Atropine crosses the placenta and traces appear in the milk.

INDICATIONS

Diphenoxylate A is used in the symptomatic treatment of acute and chronic diarrhoea. It is also used to reduce the frequency and fluidity of stools in patients with colostomies or ileostomies.

SIDE EFFECTS

The side effects of Diphenoxylate A Tablets include malaise, lethargy, sedation,

somnolence, confusion, dizziness, restlessness, depression, euphoria, hallucinations, headache, and fever.

Anaphylaxis, angioedema, urticaria and pruritus have been reported side effects.

Gastrointestinal side effects include paralytic ileus, toxic megacolon, nausea, vomiting, abdominal discomfort, constipation and anorexia.

The atropine side effects of Diphenoxylate A tablets include flushing, dryness of the skin and mucous membrane (including dry mouth), cardiac irregularities such as arrhythmias, bradycardia, and palpitations, increased intra-ocular pressure, urinary retention and difficulty in micturition, and respiratory depression in children.

Dilation of pupils with loss of accommodation, photophobia and very rarely angle closure glaucoma can occur.

PRECAUTIONS

Appropriate fluid and electrolyte therapy should be given to protect dehydration in all cases of diarrhoea. Oral rehydration therapy - which is the use of appropriate fluids including oral rehydration salts – remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative.

Drug-induced inhibition of peristalsis may result in fluid retention in the intestines, which may aggravate and mask dehydration and depletion of electrolytes, especially in young children. If severe dehydration or electrolyte imbalance is present, diphenoxylate should be withheld until appropriate corrective therapy has been initiated.

The presence of subclinical dose of atropine sulphate may give rise to the side-effects of atropine insusceptible individuals or in overdosage.

In some patients with ulcerative colitis, agents which inhibit intestinal mobility or delay intestinal transit time have been reported to induce toxic megacolon. Patients with ulcerative colitis should be observed carefully and Diphenoxylate A therapy should be discontinued promptly if abdominal distention or other untoward symptoms develop.

The medicine should be used with extreme caution in patients with advanced hepatorenal disease, and in patients with abnormal liver function, since hepatic coma may be precipitated.

Individuals with Down's syndrome appear to have increased susceptibility to the action of atropine.

Effects on Ability to Drive and Use Machines

Some of the undesirable effects such as sedation, drowsiness or dizziness may affect the ability to drive or operate machines. If affected, patients should be advised not to drive or operate machinery.

WARNING

Not recommended for children below 6 years of age.

CONTRAINDICATIONS

Diphenoxylate is contraindicated in patients with:-

- known hypersensitivity to diphenoxylate hydrochloride or atropine.
- jaundice
- intestinal obstruction
- acute ulcerative colitis
- in the treatment of diarrhoea associated with pseudomembranous enterocolitis
- raised intracranial pressure
- head injury
- myasthenia gravis
- pyloric stenosis
- paralytic ileus
- prostatic enlargement

Interactions with Other Medicaments

The following interaction have to be considered in the use of Diphenoxylate A Tablets: -

Diphenoxylate A antagonizes the effect of Domperidone, Metoclopramide, Bethanecol/ Carbachol, Cisapride. Galantamine, Neostigmine/ pyridostigmine, and Pilcarpine.

The antimuscarinic side effects of Diphenoxylate A are increased by Amantadine, Antihistamines (sedative and non-sedative), Clozapine, Disopyramide, Fluspiriline, Loxapine, MAOI's, Nefopam, Olanzapine, Phenothiazines, Quetiapine, Remoxipride, Terfenadine, Tricyclic antidepressants, and Zotepine.

Dry mouth prevents the dissolution of sublingual nitrate tablets, such as glycerol trinitrate.

The effect of Diphenoxylate A is antagonized by Dompezil.

The effect of Diphenoxylate A is increased by Memantine.

Diphenoxylate A may reduce the plasma levels of Levodopa (in combination and sole products).

Diphenoxylate A reduces the absorption of Ketoconazole.

Pregnancy and Lactation

Pregnancy

The safety of Diphenoxylate A in human pregnancy has not been established. However, as with all drugs, caution is recommended when used in early pregnancy.

Lactation

Diphenoxylate hydrochloride and atropine sulphate may be excreted in human milk. If a nursing mother is taking Diphenoxylate A, the infant may exhibit some effects of the drug.

DOSAGE AND ADMINISTRATION

Adults:

The usual initial dose is two tablets 4 times daily, later reduced when the diarrhoea is controlled.

Children:

6 to 8 years	: one tablet three times daily.
9 to 11 years	: one tablet four times daily.
Over 12 years	: two tablets three times daily.

OVERDOSAGE

Accidental overdosage may produce narcosis with respiratory depression or atropine poisoning or both, particularly in children. Symptoms of overdosage include dryness of the skin and mucous membranes, flushing, hyperthermia and tachycardia, nystagmus, pinpoint pupils, hypotonic reflexes, lethargy, coma and severe respiratory depression. The onset of symptoms of overdosage may be considerably delayed and respiratory depression may not become evident until as late as 12 to 30 hours after ingestion, and may recur in spite of an initial response to narcotic antagonists. Continuous observation should be maintained for at least 48 hours.

If respiratory depression develops, naloxone, a specific antidote should be administered. The duration of action of naloxone hydrochloride, is considerably shorter than that of diphenoxylate

hydrochloride and repeated injections of the antidote may be required. Establishment of a patent airway and artificial ventilation may be needed. If the patient is not comatose, gastric lavage and administration of slurry of activated charcoal may be indicated.

ROUTE OF ADMINISTRATION

Oral.

STORAGE CONDITIONS

Store below 30°C.

Protect from light.

Keep container tightly closed

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In bottle of 100 tablets.

In blisters of 100 tablets.

Date of revision: 5th December 2024

**PRODUCT REGISTRATION HOLDER /
MANUFACTURER:**

**PHARMANIAGA MANUFACTURING
BERHAD (198001006232)**

No. 11A, Jalan P/1, Kawasan Perusahaan
Bangi,
43650 Bandar Baru Bangi, Selangor Darul
Ehsan, Malaysia.

PRP 0006.14 051224